



# Hand Hygiene Policy

## 1. Scope

| Site                     | Operational Area | Applicable to   |
|--------------------------|------------------|-----------------|
| Armadale Kalamunda Group | All services     | All disciplines |

## 2. Purpose

Healthcare associated infections (HAI) can be acquired from multiple clinical factors, but mostly through the contaminated hands of the healthcare workers (HCW). Hand hygiene is considered the most important basic practice in preventing and controlling HAI.

The purpose of this policy is to mandate the standard practices for hand hygiene for all Health Care Workers (HCWs) across the Armadale Kalamunda Group (AKG) in accordance with the requirements of the [National Hand Hygiene Initiative](#) (NHHI).

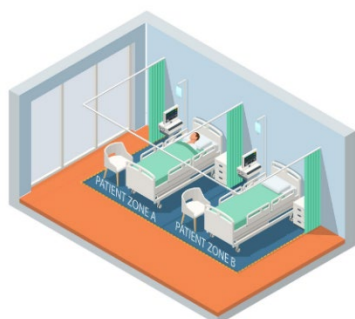
## 3. Procedure

The World Health Organisation has developed a standardised approach to education on hand hygiene. This program has been adopted nationally in Australia and is known as the 5 moments for hand hygiene.

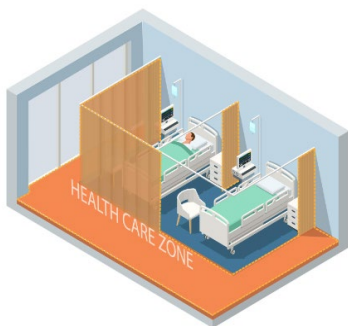
Hand hygiene is a general term referring to any action of hand cleansing. These can be achieved by either applying adequate amount of alcohol-based hand rub to the surface of the hands or washing hands with the use of a soap and water.

A moment is when there is a perceived or actual opportunity for transfer to infectious microorganisms between healthcare worker, patient, and the healthcare environment via the hands of healthcare workers.

In the 5 moments for hand hygiene approach, the healthcare environment is divided into two zones:



**Patient Zone** The patient zone is a space dedicated to an individual patient for that patient's stay. This area is cleaned between the discharge of one patient and the arrival of the next to minimise the risk of transmission of organisms between patients. Assumptions are generally made that within the patient zone the patient flora rapidly contaminates the entire patient zone; and the patient zone is cleaned between patients. Includes the patient and the patient's immediate surroundings.



**Healthcare zone-** The healthcare zone can include shared patient areas as these areas are not cleaned between patients. Assumptions are generally made that within the healthcare zone there are organisms foreign and potentially harmful to all patients, and that transmission of these pathogens to the patient results in exogenous infection.

## 4. Indications for hand hygiene

The following are indications for hand hygiene across all setting:

- Wash hands with soap and water when visibly dirty or soiled with blood or other body fluids; or after using the toilet
- If exposure to potential spore-forming pathogens is suspected or proven, including outbreaks of *C. difficile*, hand washing with soap and water is the preferred means of hand hygiene
- Use an alcohol-based hand rub for visibly clean hands and after glove use
- Do not use an alcohol-based hand rub immediately after washing with soap and water.
- Perform hand hygiene:
  - Perform hand hygiene on entering the patient zone before touching the patient
  - Immediately before a procedure. Once hand hygiene has been performed, nothing else in the patient's environment should be touched prior to the procedure starting.
  - Hand hygiene immediately after a procedure or body fluid exposure risk as hands could be contaminated with body fluid.
  - After touching a patient. Perform hand hygiene before you leave the patient zone.
  - Hand hygiene after touching a patient's surroundings even when the patient has not been touched. Always perform hand hygiene before leaving the patient's room.
  - Before handling medication or preparing food.
  - If moving from a contaminated body site to another body site during care of the same patient
  - After removing gloves

## 5. Hand hygiene techniques

### 5.1 Alcohol-based hand rub

For visibly clean hands and after removal of gloves, use alcohol-based hand rub (ABHR) as follows:

- Apply sufficient amount of ABHR product that is enough to cover all surfaces of the hands
- Rub over all surfaces for approximately 20-30 seconds until dry
- Do not rinse or wipe hands

Please see the [‘Hand Hygiene Technique with Alcohol-based Hand Rub’](#) poster.

## 5.2 Soap and water

For visibly soiled hands, wash hands as follows:

- Wet hands with warm water then apply liquid soap
- Rub hands vigorously for 40-60 seconds to produce lather
- Ensure every surface of the hand is covered
- Attention should be paid to thumbs, fingertips, and web spaces
- Rinse under warm running water then pat dry thoroughly with a paper towel
- Use dry paper towel to turn off tap if not using an elbow-operated tap.

Please see the '[Hand Hygiene Technique with Soap and Water](#)' poster.

## 5.3 Surgical hand antisepsis

- All perioperative personnel have a duty to follow a standardised procedure prior to donning sterile gown and gloves.
- Before performing surgical hand antisepsis perioperative personnel shall:
  - ensure that their skin is intact and healthy, and their fingernails are short and free of artificial product.
  - remove all jewellery below the elbow
  - report any instances of irritant and contact dermatitis
- There are two accepted techniques for performing surgical hand antisepsis (refer to the [2023 ACORN Standards](#) for procedure):
  - Surgical hand scrub
  - Alcohol-based surgical hand rub (refer to the 'Surgical hand preparation technique with an alcohol-based hand rub' poster)
- The products chosen for surgical hand antisepsis shall comply with Therapeutic Goods Administration (TGA) requirement.
- Perioperative personnel shall follow manufacturers instruction when using products chosen for surgical hand antisepsis.

## 6. Use of gloves

- Glove use is never a substitute for hand hygiene
- Non-sterile single use gloves should be worn whenever there is the possibility of contact with blood or body fluids
- Gloves must be changed between patients and between multiple procedures on the same patient where there is a risk of cross contamination
- Gloves must never be washed or have ABHR applied to them after use.
- Hand hygiene must always be undertaken on removal of gloves.
- Sterile gloves must be worn for all invasive procedures.
- Perioperative personnel shall double glove with perforation indicator system.

## 7. Bare below the elbow principles

The presence of jewellery and clothing on the hands and forearms impacts on the ability to perform hand hygiene correctly. Staff entering the clinical zone and those having direct contact with patients, must comply with the principles outlined below:

- Bracelets, wrist watches and rings with stones or ridges should not be worn when providing clinical care.
- A single flat ring/plain band may be worn but must not interfere with effective hand hygiene practice.
- Medical alert necklaces are recommended over bracelets.
- Long sleeved clothing should be rolled up above the elbow so as not to interfere with correct hand hygiene practice.
- Attire that is not laundered daily (i.e., cardigans/jumpers) must not be worn whilst providing direct patient care.
- HCWs must have clean fingernails no longer than the tip of the finger. Long nails puncture gloves more readily and are more difficult to clean.
- HCWs working in the clinical areas, including cleaning staff and food services staff, must not wear nail cosmetics/enhancements or nail polish of any type.

### **Cultural and religious considerations:**

When not engaged in patient care, some staff members may wish to cover their forearms due to religious, cultural or safety reasons. These staff must ensure they are wearing clothing with sleeves which can be pushed back securely when they enter the clinical area.

## 8. Product placement

When considering product placement, a '[Risk Assessment for Use of Alcohol-Based Hand Rubs in Healthcare Facilities](#)' can be undertaken, and a management plan put in place.

This particularly applies to clinical areas managing patients with alcohol use disorder and patients at risk of deliberate self-harm. The ABHR Risk Assessment form is available from the Australian Commission on Safety and Quality in Health Care (ACSQHC) website – [Alcohol-based hand rubs](#) page.

The following considerations apply for product placement:

- ABHR will be placed in a bracket at the foot of every bed and patient trolley (other than Mental Health) and on all treatment trolleys.
- ABHR will be placed at the ward entrance with signage to encourage visitors to perform hand hygiene on entering and leaving the ward. Availability will be monitored by the Patient Care Assistant.
- Patients who are unable to ambulate should be assisted with hand washing using soap and water or ABHR.
- Small personal bottles that HCWs carry with them may be more appropriate in some areas, such as community services settings.

## 9. Hand care

The main type of skin irritation associated with hand hygiene, irritant contact dermatitis, includes symptoms such as dryness, irritation, itching and sometimes cracking and bleeding. Allergic contact dermatitis is rare and represents an allergy, which may be to some ingredient in a hand hygiene product.

Strategies may minimise occupational hand dermatitis:

- Using of a hand hygiene product that contains skin emollient to minimise the risk of skin irritation and drying.
- Use the hand hygiene and hand care products supplied by the organisation regularly (e.g., at meal breaks and on completion of the shift) to prevent dryness.
- HCWs with cuts or abrasions should cover any breaks in skin with a waterproof dressing (e.g., transparent occlusive dressing).
- Educating staff on the correct use of hand hygiene products
- Educating staff on caring for their hands, including the regular use of skin moisturisers both at work and at home - such moisturising skin-care products need to be compatible with alcohol-based hand rub.
- HCWs who are required to wear a splint whilst at work, must discuss further with the IPM team and/or WHS to determine their ability to effectively perform HH in their work setting.
- HCWs with concerns regarding the ability to cover cuts/abrasions appropriately and/or appear sensitive to hand hygiene products or gloves, must discuss further with the IPM team and/or WHS to determine their ability to effectively perform hand hygiene in their work setting.

## 10. Involving patients and visitors

- Educate both patients and visitors about the importance of hand hygiene to minimise cross transmission in hospitals.
- Provide the patient with alcohol hand rub or a moistened, soapy wash cloth and drying towel for use by patients when access to a hand basin is not possible.
- Visitors should perform hand hygiene when visiting the patient and upon leaving the patients room.

## 11. Roles and responsibilities

| Role                      | Responsibility                                                                                                                                                                                                                                                                                                           |
|---------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Healthcare Workers (HCWs) | Have a responsibility to ensure they comply with: <ul style="list-style-type: none"><li>• 5 moments for hand hygiene</li><li>• The relevant mandatory learning requirements for hand hygiene</li><li>• The <a href="#">Dress Code – Staff, Contractors and Volunteers Standard of Clothing Policy EMHS: 52</a></li></ul> |
| Manager and Supervisors   | <ul style="list-style-type: none"><li>• Responsible for monitoring and supporting HCWs to complete the relevant hand hygiene learning requirements.</li></ul>                                                                                                                                                            |

| Role                                     | Responsibility                                                                                                                                                                                                                                                                                                                                                                                                                             |
|------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                          | <ul style="list-style-type: none"> <li>Implement the NHHI program as relevant to their size and acuity and to support credentialed and registered hand hygiene auditors where relevant, to maintain their validation training requirements.</li> <li>Results of audits and other quality improvement activities should be disseminated to relevant key stakeholders to assist in facilitating improved hand hygiene compliance.</li> </ul> |
| Infection Prevention and Management Unit | <ul style="list-style-type: none"> <li>Coordinate the hand hygiene auditing program.</li> <li>Responsible for ensuring new hand hygiene products or proposed changes to hand hygiene products, are formally evaluated through a product evaluation committee with IPM input.</li> </ul>                                                                                                                                                    |

## 12. Legislative context

The following documents are required to give effect to this policy:

- [Healthcare Associated Infection Surveillance in Western Australia Policy MP0108/19](#)
- [Infection Prevention and Management Policy EMHS: 34](#)
- [Mandatory Training Policy EMHS: 51](#)
- [Dress Code – Staff, Contractors and Volunteers Standard of Clothing Policy EMHS: 52](#)

## 13. Compliance, monitoring and evaluation

Compliance against this policy will be monitored by the Preventing and Controlling Infections Committee (PCIC) using the following strategies:

- Quarterly bare below the elbow audit to include: .
  - No long sleeves
  - No wristwatch
  - Plain ring/no ring
  - No artificial/no long nails
  - No bracelets/bangles
  - Quarterly audit with hand hygiene equipment, supplies and products requirements including: Hand washing soap available at hand basin
  - Moisturiser cream available at hand basin
  - Hand basins are free of obstacles
  - Paper towels are available at hand basin
  - Waste bin available near the hand basin
  - ABHR is available inside the room/bay
  - All hand hygiene product have not expired
  - Gloves with 3 sizes available
- Quarterly Hand hygiene audit

Hand hygiene compliance auditing is the established outcome measure for assessing the effectiveness of a hand hygiene program within the NHHI.

The NHHI hand hygiene compliance auditing method is by direct observation of HCWs by trained and validated auditors to monitor compliance with the 5 Moments for Hand Hygiene.

The following is applicable for the audit program at AKG:

- All hand hygiene auditors for the AKG must complete the required training as outlined in the Auditor Training section of the NHHI Manual (ACSQHC, 2019)
- Data collection will be completed using the Hand Hygiene Compliance Application ([HHCApp](#)) or the paper-based NHHI standardised audit tool three times annually (national audit periods) and submitted to the NHHI database
- All HCW groups will be included as part of the audit program in accordance with the specifications in the NHHI Manual (ACSQHC, 2019)
- The minimum total number of hand hygiene moments per audit for the organisation is 800 (100-200 acute inpatient beds)<sup>1</sup>
- The Infection Prevention and Management Committee (IPMC) has approved the decision for the mental health inpatient wards to not be included as part of the NHHI hand hygiene compliance auditing program; however, glove use and product placement should still be audited monthly.
- The Central Sterile Supply Department (CSSD), kitchen, laundry, and other areas where there are no patients are not to be included as part of the NHHI hand hygiene compliance audit program.

## 14. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 3 – Action 3.10: The health service organisation has a hand hygiene program that is incorporated in its overarching infection prevention and control program as part of standard precautions and:
  - Is consistent with the current National Hand Hygiene Initiative, and jurisdictional requirements
  - Addresses noncompliance or inconsistency with the current National Hand Hygiene Initiative.
  - Provides timely reports on the results of hand hygiene compliance audits, and actions in response to audits, to the workforce, the governing body, consumers, and other relevant groups.
  - Uses the results of audits to improve hand hygiene compliance.

## 15. References

1. [Australian Commission on Safety and Quality in Health Care. National Hand Hygiene Initiative Manual. Sydney: ACSQHC; 2019](#)
2. [Australian Guidelines for the Prevention and Control of Infection in Healthcare, Canberra: National Health and Medical Research Council \(2019\).](#)
3. [Australian College of Perioperative Nurses Ltd \(ACORN\). 2023 Standard for Safe and Quality Care in the Perioperative Environment for Organisations. Adelaide, South Australia](#)
4. [World Health Organization and WHO Patient Safety. \(2009\). WHO guidelines on hand hygiene in health care: a summary. World Health Organization](#)



## 16. Glossary

The following definitions are relevant to this policy.

| Term                                    | Definition                                                                                                                                                                                                                                                                                                             |
|-----------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Alcohol-based hand rub (ABHR)           | An alcohol-containing preparation (gel, foam, or liquid) designed for reducing the number of viable microorganisms on dry, unsoiled hands.                                                                                                                                                                             |
| Hand hygiene                            | A general term applying to processes aiming to reduce the number of microorganisms on hands. This includes application of ABHR to the surface of dry unsoiled hands; or use of soap/solution (plain or antimicrobial) and running water (if hands are visibly soiled), followed by patting dry with single-use towels. |
| Healthcare Associated Infection (HAI)   | Healthcare-associated infections (HAIs) are those infections that are acquired as a direct or indirect result of healthcare.                                                                                                                                                                                           |
| Healthcare Worker (HCW)                 | Any person employed or contracted either on a permanent, temporary, casual, volunteer or agency basis to deliver or support healthcare services.                                                                                                                                                                       |
| National Hand Hygiene Initiative (NHHI) | The NHHI is based on the WHO's Global Patient Safety Challenge 'Save Lives: Clean Your Hands' and adopts the '5 Moments for Hand Hygiene' framework.                                                                                                                                                                   |

## 17. Policy owner

Enquiries relating to this policy may be directed to:

Title: Clinical Nurse Specialist  
Department: Infection Prevention Management  
Email: [AHSinfection.controlnurses@health.wa.gov.au](mailto:AHSinfection.controlnurses@health.wa.gov.au)

## 18. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

| Version     | Effective from | Effective to | Amendment(s)                      |
|-------------|----------------|--------------|-----------------------------------|
| Version 4.1 | 31/10/2023     | 31/10/2026   | Schedule review – minor amendment |

## 19. Authorisation

This policy has been authorised and issued by the Director Nursing and Midwifery.

|                    |                                                  |
|--------------------|--------------------------------------------------|
| Authorised by      | Helen Fullarton – Director Nursing and Midwifery |
| Authorisation date | 31/10/2023                                       |
| Published date     | 31/10/2026                                       |